

FOOD METHODOLOGY
ABRIDGE

Nursing: How We Think About Value

A transparent framework for understanding where
value lives when there's no billing relationship

Every assumption is visible. Every calculation is transparent.

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THE CONTEXT

Nursing is the hardest setting to model ROI—and the most important to get right.

In outpatient medicine, a physician saves 4 minutes per visit, and you can trace a path to wRVU lift or capacity expansion. Nurses don't bill. They don't generate wRVUs. And yet nursing documentation burden is massive—25–35% of every shift spent on flowsheets, assessments, handoffs, and charting.

So where does the value live?

TWO VALUE CATEGORIES

LABOR ECONOMICS

Overtime, retention, and agency spend. Real dollars that show up in the budget. When nurses spend less time documenting, they finish shifts on time, burn out less, and the organization needs fewer travel nurses.

Direct, measurable

CARE QUALITY ENABLEMENT

Falls, pressure injuries, patient satisfaction. Influenced by bedside time. But the causal chain is indirect—documentation supports care, it doesn't replace it.

Potential value (shown separately)

We model both—but we're honest about what's directly measurable versus what we only enable. Labor economics value is defensible in CFO conversations. Care quality potential is real but requires clinical practice to realize.

THE HONEST LIMITS

WHAT WE MEASURE

- Direct attribution
- OT hours pre/post
 - Doc time per shift
 - Shift completion
 - Chart timing

WHAT WE INFLUENCE

- Indirect attribution
- Turnover rates
 - Agency utilization
 - Nurse satisfaction
 - Burnout indicators

WHAT WE ENABLE

- Supportive only
- Falls prevention
 - HAPI prevention
- Documentation creates visibility.
Clinical teams act on it.

- HCAHPS improvement
- Clinical outcomes

CONNECTED VALUE

When nurses document thoroughly and in real-time, it directly impacts inpatient revenue. Nursing assessments capture clinical indicators (CC/MCC) that support DRG accuracy and stronger payer appeals. These benefits are quantified in the Inpatient methodology.

VALUE MECHANISMS

How Time Saved Becomes Value

LABOR ECONOMICS

Overtime Reduction

Measurable

When nurses document faster, they finish on time. Not all time saved reduces OT—some goes to care—but a measurable portion does.

$\text{Hours saved} \times \text{Conversion (15–40\%)} \times \text{OT rate (1.5\times)}$

Retention Savings

Influenceable

Documentation burden → burnout → turnover. Reducing burden helps retain nurses who would otherwise leave.

$\text{FTEs} \times \text{Turnover} \times \text{Burnout\%} \times \text{Impact (10–25\%)} \times \text{Replacement cost}$

Agency Labor Reduction

Tied to retention

When nurses leave, hospitals fill gaps with agency nurses at 2–3× the cost. Better retention directly reduces agency dependency.

$\text{Nurses retained} \times \text{Coverage weeks (8–16)} \times \text{Weekly premium (\$2–4K)}$

CARE QUALITY ENABLEMENT

HAPI Prevention

Potential

Real-time documentation ensures skin assessments and risk factors are captured—enabling earlier intervention for pressure injuries.

$\text{Current HAPIs} \times \text{Doc-preventable (5\%)} \times \text{Cost/HAPI}$

Falls Prevention

Potential

Real-time fall risk scores and mobility status enable earlier preventive action. CMS does not reimburse for hospital-acquired fall injuries.

$\text{Current falls} \times \text{Doc-preventable (5\%)} \times \text{Cost/fall}$

Patient Experience (HCAHPS)

Directional

More bedside time correlates with higher scores. But HCAHPS is influenced by dozens of factors. Track as indicator, not outcome.

Top quartile HCAHPS ≈ 2% higher VBP reimbursement

The link between documentation and care quality is indirect. We don't cause fewer falls—we enable the visibility that helps prevent them. Clinical practice matters more than documentation.

TRANSPARENCY

Every Number Has a Source

We don't hide assumptions. If your data is different, the model adapts.

KEY ASSUMPTIONS

ASSUMPTION	RANGE	DEFAULT	SOURCE
Time saved/shift	15–30 min	20 min	Customer data
OT conversion	15–40%	25%	Studies
Nurse turnover	15–25%	18%	NSI 2024
Burnout-related %	30–50%	40%	ANA
Retention impact	10–25%	15%	Conserv.
Replacement cost	\$40–65K	\$52K	NSI 2024
Agency weeks	8–16 wks	12 wks	Industry
Agency premium	\$2–4K/wk	\$2,500	Travel
Doc-preventable %	3–8%	5%	Conserv.
Cost per HAPI	\$10–50K	\$26K	AHRQ
Cost per fall	\$3–30K	\$6.5K	CMS

We'd rather show a smaller number you can defend than a larger number that falls apart under scrutiny.

VALIDATION PATH

Our projections are starting points. The real answers come from your data.

TIME SAVINGS Before: • Survey nurses on doc time; review EHR session data After: • Repeat; compare with utilization data Timeline: 2–4 weeks	OVERTIME Before: • Baseline OT hours per unit/month; note seasonal patterns After: • Track Abridge vs control units Timeline: 2–3 months
RETENTION Before: • Baseline turnover by unit; exit interview data After: • Track Abridge vs control; survey on satisfaction Timeline: 12–18 months	CARE QUALITY Before: • Baseline HAPI/falls rates; baseline HCAHPS scores After: • Track Abridge vs control; be cautious on attribution Timeline: Treat as bonus, not promise.

This methodology reflects Abridge's approach to modeling nursing ROI. All defaults are conservative and editable. The goal is a defensible framework, not a predetermined answer.